

HHS AI RFI - Responses to 10 Specific Questions

Submitted by: Health Universe, Inc.

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About Health Universe: We are one of fewer than 10 platforms nationally holding ONC certification for Decision Support Interventions (§170.315(b)(11)). Our FHIR-native platform enables developers to build, deploy, and scale clinical AI applications with embedded compliance and interoperability.

QUESTION 1: What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

A. Regulatory Uncertainty

The 21st Century Cures Act's four-factor test for CDS exclusion remains poorly understood. Developers cannot determine whether FDA oversight applies, causing investment hesitation and either over-engineering for anticipated FDA review or under-investing in safety validation.

ONC Role: Work with FDA to publish joint guidance with worked examples categorizing common AI use cases into clear regulatory pathways.

B. Data Access and Interoperability Fragmentation

Despite FHIR adoption progress, implementation varies widely across EHR vendors. Startups face \$50K-\$500K integration costs per EHR vendor, favoring large companies with resources for multiple integrations.

ONC Actions Needed:

- Strengthen standards conformance, by mandating FHIR R4 conformance testing with public compliance dashboards
- Expand USCDI to include: social determinants of health (housing, food security, transportation), wearable device data, patient-reported outcomes (PROMIS, PHQ-9), medication adherence data
- Clarify information blocking provisions (45 CFR Part 171) to protect AI developers' data access rights

C. Compliance Cost Concentration

Every AI developer must independently achieve ONC certification (\$200K-\$500K), HIPAA compliance (\$100K-\$300K), and SOC 2 attestation (\$50K-\$150K). Total costs of \$500K-\$2M create barriers favoring wellfunded companies. Combined with delay tactics for integration, many smaller companies with good ideas never come to market. In addition, the architecture of Health IT is beginning to change – platforms can perform many of the functions of integration and compliance and integrate with legacy EHR vendors and innovative new start-ups.

ONC Solution: Establish platform certification framework in 45 CFR § 170.550 where certified platforms extend compliance to applications built on their infrastructure.

As a B.11-certified platform, we enable developers to inherit our certification, reducing their costs by 75%+. Formalizing this model would accelerate innovation while maintaining quality oversight.

D. Lack of Standardized Evaluation

No consensus exists on AI validation methodology. Healthcare organizations struggle to compare AI systems objectively, leading to paralysis or poor decisions based on sales pitches rather than evidence. This is true of existing LLMs and becomes more complex with MCP and agentic AI systems.

ONC Action: Establish standard AI performance reporting template (sensitivity, specificity, PPV, NPV, AUC stratified by demographics) recommended for certified AI systems, with public dashboard for comparison.

QUESTION 2: What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why?

Priority 1: Establish Platform Certification Framework

Change: Add provisions to 45 CFR § 170.550 recognizing platform certification with compliance inheritance. This is a logical extension of modular certification but creates new opportunities for new technology to be integrated into EHR systems that lack AI infrastructure.

Mechanism: Certified platforms maintain comprehensive ONC certification; applications built on certified platforms inherit compliance status; ONC audits platforms rather than individual applications.

Impact: Reduces developer costs from \$500K-\$2M to \$50K-\$200K, enabling startups to compete while maintaining oversight.

Priority 2: Information Blocking Safe Harbors for AI

Changes to 45 CFR § 171.202:

- Protect AI training data requests when data is de-identified per 45 CFR § 164.514(b)
- Protect algorithm validation for quality improvement under 45 CFR § 164.506(c)(4)
- Protect performance monitoring for bias/safety detection, including logs and other analytic tools

Impact: Enables AI development while maintaining privacy. Healthcare organizations limit sharing of data because of privacy concerns. Strengthening safe harbors and clarify the rules will increase transparency and collaboration between health systems and AI developers.

Priority 3: Mandate Consistent FHIR Implementation

Changes to 45 CFR § 170.315:

- Require conformance testing demonstrating full FHIR R4 implementation
- Mandate CDS Hooks or FHIR Clinical Reasoning for decision support integration
- Public dashboards showing EHR vendor compliance

Impact: Standardized implementation enables AI developers to build once and deploy everywhere, reducing integration costs.

Priority 3: Preserve Optional Certification for Decision Support

ONC Should: Maintain §170.315(b)(11) as voluntary certification rather than eliminating it. Make certification valuable through:

- Recognition in payment policies
- Streamlined pathways for certified platforms
- Public performance registries for certified AI

Rationale: Complete elimination removes objective quality signals healthcare organizations use to evaluate AI platforms. Voluntary certification preserves market differentiation while reducing mandatory burden.

QUESTION 3: What novel legal and implementation issues exist for non-medical device AI and what role should HHS play?

A. Liability Allocation

Issue: Unclear liability when AI generates incorrect recommendations - developer, platform, healthcare organization, or clinician?

HHS Role: Issue guidance on standard of care for AI-augmented practice and convene stakeholders to develop model liability frameworks.

B. Privacy Beyond HIPAA

Issue: AI infers sensitive information not explicitly documented (genetic predisposition from family history, mental health from behavioral patterns).

ONC Role: OCR guidance clarifying whether AI-inferred information qualifies as PHI under 45 CFR § 164.501.

C. Algorithmic Bias

Issue: AI may perpetuate health disparities through biased algorithms.

ONC Actions:

- Establish maximum allowable performance disparities (e.g., sensitivity must not differ by >5% across racial groups)
- Require bias impact assessments for certified AI
- Mandate performance reporting stratified by demographics

D. Data Governance for AI Training

Issue: Healthcare organizations uncertain about legal basis for sharing data for AI development (research vs. quality improvement vs. information blocking).

HHS Role: Unified guidance clarifying that de-identified data per Safe Harbor is not human subjects research and AI development doesn't trigger information blocking.

E. Intellectual Property vs. Transparency

Issue: Balance between transparency (clinical trust) and IP protection (innovation incentive).

ONC Role: Establish minimum transparency standards protecting trade secrets:

Require disclosure: Training data characteristics, intended use, validation performance, limitations

Protect: Specific algorithms and proprietary training data

Public reporting: Aggregate performance and bias metrics

QUESTION 4: What are the most promising AI evaluation methods for non-medical devices? Should HHS support these processes?

Pre-Deployment:

A. Prospective Clinical Validation

- Performance metrics stratified by demographics
- Minimum sample sizes appropriate to risk ($n \geq 100$ simple, $n \geq 1000$ complex)
- Confidence intervals and subgroup analysis

B. Fairness Assessment

- Multiple metrics: demographic parity, equalized odds, calibration
- Maximum 5% performance difference across protected groups

C. Robustness Testing

- Performance with missing data, outliers, distributional shifts

- Identification of failure modes

Post-Deployment:

D. Real World Performance Monitoring Given the rapidly changing ecosystem, this is the most important testing – real world performance monitoring with rapid response should there be any anomalies detected.

- Monthly performance metrics
- Data drift detection
- Adverse event surveillance
- Performance within 10% of validation

ONC Support Mechanisms:

- 1. Public Performance Registry** Voluntary registry for certified platforms demonstrating ongoing quality. Participants report quarterly metrics, adverse events, bias assessments. Create safe harbor for good faith reporting.
 - 2. Benchmark Dataset Development** Partner with healthcare organizations to create de-identified validation datasets for common use cases (sepsis, readmission, imaging).
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QUESTION 5: How can HHS best support private sector activities to promote innovative and effective AI use in clinical care?

A. Federally Recognized AI Application Marketplace

Concept: Industry-led marketplaces with federal endorsement (not operation).

Trust Seals:

- Platform Certified: Built on ONC-certified platform
- Clinically Validated: Peer-reviewed evidence or Real World Testing
- FDA Cleared: Medical device clearance

ONC Role:

- Recognize certification in payment policies
- Avoid creating new mandatory requirements

Critical Design: Marketplace displays certification status, preserving competitive value of certified technology

B. Industry Testing Standards

Support professional organizations (AMIA, ACR, CAP) developing testing protocols:

- Standardized metrics by clinical domain

- Benchmark datasets
- Best practices for validation

ONC Role: Incorporate industry benchmarks into certification criteria; recognize standards in guidance.

C. Organizational Accreditation

Extend Joint Commission model to AI developers - assess quality management systems:

- ISO 13485 equivalent
- Bias assessment methodology
- Clinical safety framework Interoperability standards

ONC Role: Streamlined certification requirements for accredited developers.

D. Private Sector Performance Registry

Structure:

- Quarterly performance metrics stratified by demographics
- Adverse event reports
- Version tracking
- Public dashboard

ONC Support:

- Technical infrastructure funding
- Recognition in payment models
- Safe harbor for reporting

QUESTION 6: Where have AI tools met or exceeded expectations and where have they fallen short? What novel AI tools have greatest potential?

Success Stories:

A. Ambient Clinical Documentation (Nuance DAX, Suki)

- 40-50% documentation time reduction
- High physician satisfaction (>80%)
- ROI within 6-12 months
- Success factors: Clear use case, immediate benefit, low patient harm risk

B. Radiology Screening (FDA-cleared AI)

- Sensitivity >95% for critical findings
- 30-40% faster time-to-diagnosis

- Reduced radiologist fatigue
- Success factors: Objective task, FDA clearance, augments not replaces

C. Clinical Trial Matching

- 3-10x increase in patients screened
- 60-70% coordinator time reduction
- Improved trial diversity
- Success factors: High unmet need, low risk

Disappointments:

D. Sepsis Prediction

- High false positives (>50%) causing alert fatigue
- Poor generalization across hospitals
- Limited evidence of outcome improvement
- Lessons: Need continuous recalibration, local validation, actionable interventions

E. Hospital Readmission Prediction

- AUC 0.70-0.75 (modest improvement)
- Prediction without intervention has limited value
- Missing key predictors (social determinants not in EHR)
- Lessons: Must couple prediction with effective interventions

High-Potential Novel Tools:

F. LLMs for Documentation Quality and Administrative Tasks

- Real-time review for completeness, coding accuracy, clinical reasoning gaps
- Could reduce denials (5-10% revenue improvement) and improve quality measures
- Barrier: Regulatory uncertainty about medical device status

G. Continuous Remote Monitoring

- AI analysis of wearables, smart home sensors, patient-reported symptoms
- Early detection of deterioration, medication adherence support
- Barrier: Lack of reimbursement for non-FDA-cleared devices

H. AI for SDOH Screening and Intervention

- Screen for social needs, connect to resources, optimize allocation
- Address root causes of disparities

- Barrier: Lack of structured SDOH data in EHRs

QUESTION 7: Which decision makers have most influence on AI adoption? What are primary administrative hurdles?

Key Decision Makers:

A. Chief Medical Information Officers (CMIOs) - Most Influential

- Bridge clinical needs and technology
- Criteria: Clinical evidence, workflow integration, physician support

B. Chief Financial Officers (CFOs) - Budget Authority

- Criteria: ROI <24 months, reimbursement clarity, revenue impact
- Current barrier: Lack of CPT codes creates uncertainty and makes ROI more challenging

C. Chief Information Officers (CIOs) - Technical Feasibility

- Criteria: API compatibility, security, vendor stability

Administrative Hurdles:

1. Procurement Complexity (6-18 months)

- Legal review of contracts, BAA negotiations, multi-stakeholder approvals, multiple IP reviews
- ONC solution: Model contracts and BAAs for AI vendors; streamlined review for certified platforms

2. EHR Integration

- Custom work for each vendor (\$50K-\$200K per EHR)
- ONC solution: Mandate standardized FHIR R4 and CDS Hooks; require conformance testing

3. Validation Uncertainty

- No standard methodology, difficult to compare vendors
- ONC solution: Standard performance metrics, public dashboards, benchmark datasets

4. Data Governance

- Uncertainty about data sharing legality
- ONC solution: Clear guidance on AI training data, information blocking safe harbors

How Platform Certification Accelerates:

Certified platforms would reduce timeline from 3-6 years to 12-18 months through:

- Pre-negotiated contracts and BAAs
- Proven EHR integration via FHIR
- Validated security and compliance
- Rapid deployment capability

QUESTION 8: Where would enhanced interoperability widen market opportunities and accelerate AI development?

Critical Data Types for USCDI Expansion:

A. Social Determinants of Health

- Housing status, food security, transportation, social isolation, Area Deprivation Index
- Essential for: Readmission prediction, chronic disease management, health equity AI
- Market impact: Enables community-based organization integration

B. Patient-Reported Outcomes

- PROMIS measures, PRO-CTCAE, PHQ-9, GAD-7, functional status
- Essential for: Treatment response prediction, symptom management
- AI Market impact: Patient engagement platforms can integrate

C. Wearable Device Data

- Continuous heart rate/HRV, activity, sleep, glucose, blood pressure
- Essential for: Remote monitoring AI, early warning systems
- Market impact: Consumer health AI at scale

D. Medication Adherence

- Pharmacy fill dates, MPR, PDC, gaps in therapy
- Essential for: Treatment response prediction, adherence interventions
- Market impact: \$300B non-adherence problem

E. Genomic Data

- Structured variants (HGVS notation), pharmacogenomic implications, tumor molecular features
- Essential for: Precision medicine AI
- Market impact: Oncology treatment selection

F. Nationally recognized guidelines

- Standardized or computable access to authoritative guidelines
- Less reliance on LLMs and incorporation of high quality peer-reviewed information into the clinician's workflow

Data Standards Needed:

G. Standardized AI Performance Metrics

- Public dashboard for comparison
- Public deidentified datasets for gold standard assessments and comparison
- Impact: Enables objective procurement decisions

H. Fairness Assessment Standards

- Multiple metrics with disclosure of tradeoffs
- Maximum allowable disparities (5% for high-stakes)
- Impact: Equity-focused organizations can select unbiased AI

I. FHIR Clinical Reasoning Adoption

- Mandate CDS Hooks or FHIR Clinical Reasoning for certified DSI
- Deprecate proprietary integration methods
- Impact: Reduces integration costs dramatically, enables platform models

Market Opportunities:

Expand innovation to independent, third-party apps stores that increase distribution channels for innovators, while providing more access to innovation for those EHR systems that serve smaller, rural and community practices.

QUESTION 9: What challenges do patients wish AI to address? What concerns do they have?

Patient Priorities:

A. Access to Care

- Wait times for appointments (weeks to months)
- AI solutions: Virtual triage, optimized scheduling, 24/7 chatbots

B. Care Coordination

- "Nobody talks to each other" - repeating history, duplicate tests
- AI solutions: Automated care summaries, medication reconciliation, care gap alerts

C. Understanding Medical Information

- Confusing terminology, limited provider time for questions

- AI solutions: Plain language translation, visual explanations, personalized education

D. Medication Management

- Complex regimens, forgetting doses, unclear side effects
- AI solutions: Smart reminders, adherence monitoring, interaction checking

E. Cost Transparency

- Unpredictable out-of-pocket costs
- AI solutions: Cost estimation, financial assistance identification, prior auth support

Patient Concerns:

1. Privacy and Data Security

- Will data be sold? Can AI be hacked? Will insurers discriminate?
- Address through: Clear consent, data minimization, audit trails, federal protections, harmonization of privacy regulations (FTC, HIPAA, FIRPA, Common Rule, FDA)

2. AI Errors and Safety

- What if AI misses something important? Who's responsible?
- Address through: Human oversight always, transparent performance, error reporting, liability clarity

3. Bias and Fairness

- Will AI discriminate based on race, gender, age, socioeconomic status?
- Address through: Mandatory bias testing, diverse training data, community engagement, opt-out option

4. Loss of Human Connection

- Will AI replace my doctor?
- Address through: Emphasize AI saves physician time for patients, enhances not replaces

5. Lack of Control

- "Black box" making decisions about my health
- Address through: Plain explanations, transparency in recommendations, opt-in consent

ONC Role:

- OCR enforcement of HIPAA
 - Mandatory adverse event reporting for certified AI
 - Maximum allowable performance disparities
 - Health literacy standards for AI disclosures
 - Patient privacy bill of rights
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QUESTION 10: What AI research should HHS prioritize? What does literature show about costs, benefits, and transfers?

A. Published Findings on AI Impact:

Strong Evidence:

Radiology AI: Sensitivity >90% for specific findings; improved detection, reduced reading time

Diabetic retinopathy screening: FDA-cleared autonomous AI; cost-effective in primary care (\$500/QALY)

Pathology AI: High sensitivity for cancer detection; reduced inter-observer variability

Moderate Evidence:

- **Clinical Decision support and guideline adherence** - Mixed results - some early detection, others high false positives and poor generalization

Clinical documentation AI: 30-50% time savings, high satisfaction in pilots

Evidence Gaps:

- Most studies report accuracy, not patient outcomes (mortality, quality of life)
- Limited cost-effectiveness analyses
- Sparse implementation science literature
- Few studies assess health equity impact

B. Literature on Costs, Benefits, Transfers:

Implementation Costs (Well-Documented):

- AI licensing: \$10K-\$500K annually
- Integration: \$50K-\$500K
- Training: \$25K-\$100K
- Ongoing maintenance: 15-20% annually

Cost Savings (Mixed Evidence):

- Time savings: 20-50% depending on application
- Revenue enhancement: 5-10% from improved coding
- Cost avoidance: Limited evidence for avoided hospitalizations

Value Transfers:

- From payers to providers if AI reduces costs but payments unchanged
- From providers to AI vendors through subscription fees
- From physicians to organizations if productivity gains not shared
- Risk of widening digital divide if AI only accessible to well-resourced organizations

Research Priorities for ONC:

1. Real World Performance Standards ONC should establish requirements for post-deployment monitoring:

- Quarterly performance reports stratified by demographics
- Adverse event reporting within 48 hours
- Performance degradation triggers (>10% from validation)
- Incentivize the development of standardized, annotated datasets for gold standard assessments and comparisons.

2. Interoperability Standards Development ONC support for HL7 development of:

- Model card format and content standards
- AI performance reporting templates
- CDS Hooks extensions for AI integration
- FHIR resources for AI metadata

3. Bias Assessment Methodologies ONC should define:

- Standard fairness metrics for healthcare AI
- Maximum allowable performance disparities
- Reporting requirements for certified AI
- Mitigation strategies when thresholds exceeded
- Datasets that can benchmark and provide gold standard validation for AI tools.

4. Validation Dataset Development ONC coordination of multi-institutional efforts to create de-identified benchmark datasets for common AI use cases.

CONCLUSION

Health Universe urges ONC to:

1. Establish platform certification framework enabling compliance inheritance
2. Preserve certification as optional but valuable market differentiator
3. Mandate consistent FHIR implementation with public dashboards
4. Expand USCDI with AI-essential data elements
5. Create information blocking safe harbors for AI development
6. Create standardized datasets with annotations for gold standard assessments and benchmarking
7. Require bias impact assessments for certified AI

These actions would accelerate beneficial AI innovation while maintaining patient safety and trust.

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